

Women's Specialists of Clear Lake

Dr. Geoffrey H. Klein, MD · FACOG · DABOM · 400 W Medical Center Blvd, Suite 300, Webster, TX 77598 · (281) 557-0300

Common Breastfeeding Problems

Most breastfeeding difficulties can be overcome with the right support — don't give up, and don't hesitate to ask for help

IS MY BABY GETTING ENOUGH MILK?

- ✓ By day 5: at least **6–8 wet diapers** and **3+ stools** daily
- ✓ Newborns feed **8–12 times** per 24 hours; by 4 weeks, **7–9 times**
- ✓ Some weight loss is normal — up to **7%** in the first 3–5 days
- ✓ Baby should regain birth weight by **1–2 weeks** of life
- ✓ If baby seems hungrier than usual, it may be a **growth spurt** (2–4 weeks, 6 weeks, 3 and 6 months)

SORE & CRACKED NIPPLES

- Most common cause: **improper latch** — correcting position usually fixes it
- Mild sensitivity is normal the first week and resolves by **day 7–10**
- Pain that persists or worsens throughout a feeding = **nipple injury**
- Apply **antibiotic ointment** (bacitracin/mupirocin) to cracked nipples with a nonstick pad
- Lanolin is commonly used but **evidence for benefit is limited**
- Check baby for **tongue-tie** if latch problems persist despite good technique

ENGORGEMENT & BLOCKED DUCTS

- **Engorgement** (days 3–5): breasts feel full, firm, and tender as milk comes in
- Hand-express a small amount **before** feeding to soften the areola for latch
- **Blocked ducts**: tender lump without fever — feed on demand, avoid over-pumping
- Use **cold compresses** between feeds and **ibuprofen** for pain/inflammation
- Avoid deep massage or vibrating devices — they can **worsen inflammation**
- If not improved in **48 hours** or fever develops, call your doctor (may be mastitis)

MASTITIS & BREAST INFECTIONS

- **Mastitis**: red, painful, swollen area of breast, often with fever and flu-like symptoms
- Most common in the **first 3 months** postpartum
- Continue breastfeeding — stopping can make it **worse**
- Antibiotics may be needed if symptoms don't improve in **24–48 hours**
- A fluctuant, tender mass may indicate a **breast abscess** (needs drainage)
- "Candidal" breast infections are **often overdiagnosed** — testing can confirm

LOW MILK SUPPLY

- ✓ Most common fix: **more frequent nursing** with proper latch technique
- ✓ Milk typically "comes in" by **72 hours** — delayed in ~20% of women
- ✓ Stress, obesity, diabetes, PCOS, and some medications can affect supply
- ✓ Avoid unnecessary formula supplements — they can **reduce** milk production
- ✓ **Galactagogues** (herbal or prescription) are **not recommended** — limited evidence
- ✓ If supplementation is needed, use syringe/cup instead of bottle when possible

TIPS FOR SUCCESS

- ✓ Get a **lactation consultant** evaluation early if you're struggling
- ✓ Ensure baby's mouth is **wide open** with chin buried in breast during latch
- ✓ Nurse in a comfortable position — try **cradle, cross-cradle, football, or side-lying**
- ✓ Most medications are **safe** while breastfeeding (check LactMed database)
- ✓ Wean **gradually** — drop one session every 2–5 days to avoid engorgement

⚠️ CALL YOUR DOCTOR OR LACTATION CONSULTANT IF

- ⚠️ Baby has lost more than 10% of birth weight
- ⚠️ Fewer than 6 wet diapers per day after day 5
- ⚠️ Nipple pain that worsens or persists beyond the first week
- ⚠️ Breast redness, swelling, or fever (possible mastitis)
- ⚠️ Baby is not back to birth weight by 2 weeks of age
- ⚠️ You feel overwhelmed, sad, or anxious about breastfeeding

Breastfeeding Gets Easier — You're Doing a Great Job

Almost every breastfeeding challenge has a solution. With proper support and technique, most difficulties resolve within the first few weeks. You are not alone — our team and lactation consultants are here to help you every step of the way!